CHILDREN'S SPECIALTY HOSPITAL

EMERGENCY DEPARTMENT ENCOUNTER SUMMARY

PATIENT DETAILS:

Patient Name: Aarav Joshi | Age: 14 | Gender: Male
UHID: CSH-PED-2026-6190 | Encounter Date: 2026-10-06
Attending Pediatrician: Dr. Sanjay Mehra, MD (Pediatrics)

PRESENTING COMPLAINTS:
Sudden acute breathlessness, audible wheezing, and continuous dry hacking cough following exposure to outdoor dust and cold morning

EMERGENCY TRIAGE & VITALS:

- Respiratory Rate: 30 breaths/min (Marked tachypnea with subcostal retractions)
- Heart Rate: 118 bpm (Sinus tachycardia)
- Oxygen Saturation (SpO2): 90% on ambient room air -> Improved to 98% on 2L humidified O2 via nasal cannula
- Auscultation: Bilateral widespread high-pitched expiratory wheezes
- Peak Expiratory Flow (PEFR): Initial 180 L/min (40% predicted) -> 330 L/min (73% predicted) post-treatment.

PRIMARY DIAGNOSIS:

Acute Moderate-to-Severe Exacerbation of Bronchial Asthma triggered by environmental aeroallergens.

ACUTE RESCUE THERAPY GIVEN:
1. Serial nebulizations with Salbutamol 2.5 mg + Ipratropium Bromide 0.5 mg every 20 minutes x 3 doses.
2. Intravenous Hydrocortisone 100 mg stat.
3. Humidified supplemental oxygen.

DISCHARGE PRESCRIPTION:

1. Budesonide Inhaler (Budecort 200 mcg) - 2 puffs twice daily using spacer chamber (Daily anti-inflammatory controller).
2. Salbutamol Inhaler (Asthalin 100 mcg) - 2 puffs as needed for acute wheezing/coughing (Quick-relief rescue inhaler).
3. Tab. Prednisolone 20 mg - 1 tablet daily in the morning with breakfast for 5 days only, then discontinue (Oral steroid burst).
4. Tab. Montelukast 10 mg - 1 tablet daily at bedtime for 30 days (Leukotriene modifier).
5. Tab. Cetirizine 5 mg - 1 tablet daily at bedtime for 10 days (Antihistamine).

CRITICAL INHALER & MEDICATION CAUTIONS:

- Always gargle with water and spit out after using Budesonide inhaler to prevent oral candidiasis (fungal thrush).
- Keep Salbutamol rescue inhaler accessible in backpack at all times during school and sports.

PARENT & PATIENT GUIDANCE:

- Avoid identified triggers: Dust mites, damp carpets, tobacco smoke, cold drafts, and pollen.
- Monitor Peak Expiratory Flow daily: Yellow Zone (50-80%) requires immediate rescue puffs; Red Zone (<50%) requires immediate return to Em
- Follow up in Pediatric Pulmonology Clinic in 1 week.